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The Honorable Thomas O Rice

IN THE UNITED STATES DISTRICT COURT
FOR THE EASTERN DISTRICT OF WASHINGTON
AT SPOKANE

JANE ELIZABETH ROBERTS, *et al,*

Plaintiffs,

v.

SHRINERS HOSPITALS FOR CHILDREN, SHRINERS HOSPITALS FOR CHILDREN-SPOKANE, BEVERLY BOKOVITZ, FRANCES FARLEY, JERRY GANTT, JOHN McCABE, PHILLIP GRADY, PETER BREWER, AND GOV. JAY INSLEE

Defendants.

No. 2:23-CV-295-TOR

PLAINTIFFS' OPPOSITION TO SHRINERS CHILDREN'S MOTION TO DISMISS

2/5/2024

ORAL ARGUMENT REQUESTED

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PLAINTIFFS' OPPOSITION TO MOTION TO DISMISS (ORAL ARGUMENT REQUESTED)

Plaintiffs respectfully file this Opposition to the Motion to Dismiss filed by the Shriners Defendants as follows:

I. INTRODUCTION

Nearly all of Shriners' Motion mischaracterizes this case as being about a mandate of a licensed vaccine. It is not. "Plaintiffs make no assertions regarding whether it is lawful for a public or private entity to mandate a ***licensed*** vaccine. Plaintiffs' allegations herein only relate to Defendants' mandating the use of drugs, biologics, and devices under 21 U.S.C. §360bbb-3 (the EUA statute) and the PREP Act." (Doc 8, ¶ 20) "Plaintiffs adamantly assert that an individual has the absolute Constitutional and federal statutory right to refuse the administration of an Emergency Use Authorization (EUA) drug (e.g., Pfizer BioNTech COVID-19 Vaccine), biologic, or device (e.g., EUA testing articles and masks) or a "covered countermeasure" under PREP Act immunity without incurring a penalty or losing a benefit to which they are otherwise entitled and that such a right is not dependent upon a person seeking a religious or medical exemption." (Doc 8, ¶ 21)

Contrary to Defendants' arguments, Plaintiffs do not contest Shriners' right to issue a mandate of a licensed vaccine. Rather, Plaintiffs allege that Shriners issuing a mandate of investigational new drugs under 21 U.S.C. §360bbb-3 (the EUA

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statute) and the PREP Act violates federal law and Plaintiffs' federal and Constitutional rights.

Moreover, Shriners mischaracterizes laws governing the $600 billion pharmaceutical industry to imply that the laws applicable to "Pfizer-BioNTech COVID-19 Vaccine" (i.e., the EUA investigational drug) were no longer in effect once Comirnaty® was approved simply because the two drugs allegedly shared the same formulation at one point. Shriners' claim is a legal fiction, which now requires this court to clarify drug labeling laws and deny Shriners' Motion to Dismiss since their entire argument hinges upon this logical and legal fallacy. Thus, when analyzing the issues in this case, it is important to distinguish between the "Pfizer-BioNTech COVID-19 Vaccine," which was the available EUA/PREP Act product, and "Comirnaty®," which was the unavailable FDA-licensed product. Both drugs have completely different labels under two completely different sets of laws impacting Plaintiffs' legal rights.

II. LEGAL STANDARD

A Complaint does not require detailed factual allegations; it simply must plead "enough facts to state a claim to relief that is plausible on its face." *Bell Atlantic Corp. v. Twombly*, 550 U.S. 544, 570 (2007). When considering a Rule 12 Motion to Dismiss, the Court is required to accept Plaintiffs' factual allegations as true. *Ashcroft v. Iqbal,* 556 U.S. 662 (2009).

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III. SHRINERS' LOGICAL FALLACIES

Shriners Defendants build their defense around four logical fallacies: (1) they are not State Actors, (2) drugs are governed by their formulation and not their labeling, (3) the EUA statute and PREP Act are not enforceable under § 1983, (4) they have a right to issue a mandate relying on EUA/PREP Act drugs. Therefore, Plaintiffs will speak to these fallacies up front, resulting in a shorter response.

IV. STATE ACTOR

Shriners Defendants' claim that "Shriners Children's is not a state actor subject to 42 U.S.C. § 1983" is incorrect. (Doc 32, p. 12) "Faithful adherence to the 'state action' requirement of the Fourteenth Amendment requires careful attention to the gravamen of the plaintiff's complaint." *Blum v. Yaretsky*, 457 U.S. 991, 1003 (1982). The gravamen of Plaintiffs' Complaint is stated in clear, unambiguous language, "Plaintiffs' allegations herein only relate to Defendants mandating the use of drugs, biologics, and devices authorized under 21 U.S.C. §360bbb-3 and the PREP Act." (Doc 1, ¶ 190).

As the Ninth Circuit held in *Johnson v. Duffy*, 588 F.2d 740, 743 (9th Cir. 1978). "Section 1983 provides, in pertinent part, that '[e]very person who, under color of any statute of any state . . ., subjects, or causes to be subjected, any…person within the jurisdiction thereof to the deprivation of any rights, privileges, or immunities secured by the Constitution and laws, shall be liable to the party

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injured…'" Because Shriners Defendants were acting under color of state law when it acted at the direction of Gov. Inslee's mandate, this action is for the recovery of damages from Defendants that resulted from the deprivation of Plaintiffs' federal statutory and Constitutional rights by a state actor.

A. State Action Background

As Plaintiffs' Complaint alleges (Doc 1, ¶ 172), all EUA/PREP Act COVID-19 drugs distributed in the United States were purchased by the federal government. According to the products' labeling, they were not FDA-licensed as a vaccine against COVID-19, thus the need for an emergency use authorization. Therefore, because the drugs met the definition of "investigational" under the federal statutory scheme (see Doc 1, Footnote 109 and discussion *infra*), the federal executive branch was bound to 45 CFR Part 46, the Belmont Report, Article VII of the ICCPR Treaty, 21 U.S.C. §360bbb-3, 10 U.S.C. § 980, the federal constitution, and the PREP Act, among other laws. The federal executive branch established the CDC COVID-19 Vaccination Program Provider Agreement (hereinafter referred to as "Program") so that these federally owned EUA/PREP Act products could be lawfully offered to the public. The CDC states that "this program is a part of collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC. To receive one or more of the publicly funded COVID-19 vaccines (COVID-19 Vaccine), constituent products, and ancillary supplies at no cost, Organization agrees

that it will adhere to the following requirements…" (Emphasis added). The CDC relied upon the voluntary participation of States because each state already had a Federal Wide Assurance[1] agreement in place requiring compliance with the same laws as a condition upon the State to access federally funded products.

Governor Inslee volunteered the State of Washington to participate in the Program and abide by its terms and conditions. The Washington State Department of Health established guidelines on how private parties could participate with the State to distribute the federally funded property. If Shriners wanted to participate as an Organization under the Provider Agreement, its CEO and CMO had to sign the contract, and the Program required the following:

> (1) Organization must administer COVID-19 Vaccine in accordance with all requirements and recommendations of CDC and CDC's Advisory Committee on Immunization Practices (ACIP),
>
> (2) Within 24 hours of administering a dose of COVID-19 Vaccine and adjuvant (if applicable), Organization must record in the vaccine recipient's record and report required information to the relevant state, local, or territorial public health authority,
>
> (3) Organization must submit Vaccine-Administration Data through either (a) the immunization information system (IIS) of the state and local or territorial jurisdiction or (b) another system designated by CDC according to CDC documentation and data requirements,
>
> (4) Organization must not sell or seek reimbursement for COVID-19 Vaccine and any adjuvant, syringes, needles, or other constituent

[1] The State of Washington operates under FWA00000327.

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products and ancillary supplies that the federal government provides without cost to Organization,

(5) Organization must administer COVID-19 Vaccine regardless of the vaccine recipient's ability to pay COVID-19 Vaccine administration fees,

(6) Before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) fact sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative,

(7) Organization's COVID-19 vaccination services must be conducted in compliance with CDC's Guidance for Immunization Services During the COVID-19 Pandemic for safe delivery of vaccines,

(8) Organization must comply with CDC requirements for COVID-19 Vaccine management,

(9) Organization must report moderate and severe adverse events following vaccination to the Vaccine Adverse Event Reporting System (VAERS),

(10) Organization must provide a completed COVID-19 vaccination record card to every COVID-19 Vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative. Each COVID-19 Vaccine shipment will include COVID-19 vaccination record cards,

(11) Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws,

(12) Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA (under "conditions of authorization" in the EUA letter) that covers COVID-19 Vaccine.

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As Plaintiffs alleged (Doc 1, ¶¶ 107, 108, 166, 167, 219, 224, 264), the HHS Secretary is the only person authorized by Congress to authorize emergency use of unlicensed drugs and to establish the conditions under which such use can occur. The Secretary issues an EUA letter containing the "Conditions of Authorization" to the respective manufacturer of the authorized product. (Doc 1, ¶¶ 79-81). As an example, after the FDA licensed Comirnaty®, the agency nevertheless issued an EUA for "Pfizer-BioNTech COVID-19 Vaccine" stating that "Emergency response stakeholders will identify vaccination sites to receive authorized Pfizer-BioNTech COVID-19 Vaccine and ensure its distribution and administration, consistent with the terms of this letter and CDC's COVID-19 Vaccination Program."[2] The HHS Secretary defined an "emergency response stakeholder"[3] as "a public health agency and its delegates that have legal responsibility and…authority to administer, deliver, or distribute vaccine in an emergency situation." The HHS defined a "vaccination provider" as "the facility, organization, or healthcare provider licensed or otherwise authorized by the emergency response stakeholder (e.g., non-physician healthcare professionals, such as nurses and pharmacists pursuant to state law under a standing order issued by the state health officer) to administer or provide vaccination services

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[2] See Exhibit A, Letter O

[3] See Exhibit A, FN 12

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in accordance with the applicable emergency response stakeholder's official COVID-19 vaccination and emergency response plan(s) and who is enrolled in the CDC COVID-19 Vaccination Program."[4] Therefore, the Governor and Shriners Defendants ("Organization") were lawfully obligated to ensure that individuals considering the use of the federally funded property were informed of their lawful right to accept or refuse the product without consequence. Moreover, since the Program was operating under emergency conditions under the State's prerogative, and the products were federally funded, the Program must meet the stringent threshold of protecting individuals' Fourteenth Amendment rights. The State distributed the COVID-19 property on behalf of the federal government and then recruited private parties like Shriners to perform the State's duties on its behalf, including obtaining an individual's legally effective informed consent and conducting research activities required of them by the federal government. (Doc 1, Section D, p. 19)

The Governor's office, through the Washington State Department of Health, established a process to recruit and identify vaccination providers. Once an entity was authorized by the State and the CDC, the HHS Secretary established the following mandatory condition:

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[4] See Exhibit A, FN 13

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> Emergency response stakeholders will ensure that vaccination providers within their jurisdictions are aware of this letter of authorization, and the terms herein and any subsequent amendments that might be made to the letter of authorization, **instruct them** about the means through which they are to obtain and administer the vaccine under the EUA, and ensure that the authorized labeling [i.e., Fact Sheet for Healthcare Providers Administering Vaccine (Vaccination Providers) and Vaccine Information Fact Sheet for Recipients and Caregivers] is made available to vaccination providers through appropriate means (e.g., e-mail, website).[5]" (Emphasis added).

The HHS Secretary established requirements for "vaccination providers" to conduct training, report adverse events, conduct patient follow-ups requested by the U.S. Government, "comply with CDC and/or emergency response stakeholder vaccine management requirements," and maintain administration records.[6]

However, the statutory conditions referenced above for Governor Inslee and those private parties he recruited to help the State distribute the federally owned property are in addition to the conditions required of the Program.

The State incorporated the CDC Vaccination Program Provider Agreement into its requirements as state policy when requiring any party to sign that agreement as a condition to engage in joint activity with the state. Therefore, the Agreement might have been written and secured by the CDC, but the terms and

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[5] See Exhibit A, Letter P

[6] See Exhibit A, Letters R-W

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conditions were state policies since the signing of the contract was a condition of the State upon Defendants as a right to join it in distributing the federal property.

Upon agreeing to volunteer the State of Washington to distribute the federal government's COVID-19 property, Governor Inslee was then bound to apply the laws associated with the Program in accordance with the Fourteenth Amendment's fundamental Equal Protection, Due Process, Right to Privacy, and Excessive Fines Clause doctrines. Governor Inslee might have been empowered by the State Legislature to delegate the state function of distributing the federal COVID-19 property through authorized private parties, but he was not authorized to delegate that state function without also delegating the Constitutional obligations.[7] Therefore, the State cannot abdicate its statutory and Constitutional duties by delegating its

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[7] In *Giron v. Corrections Corp. of America*, 14 F. Supp. 2d 1245 (D.N.M. 1998), the court stated, "If a state government must satisfy certain constitutional obligations when carrying out its functions, it cannot avoid those obligations and deprive individuals of their constitutionally protected rights by delegating governmental functions to the private sector. See *Terry v. Adams*, 345 U.S. 461, 73 S. Ct. 809, 97 L. Ed. 1152 (1953). The delegation of the function must carry with it a delegation of constitutional responsibilities."

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functions to private parties. The State was not under lawful obligation to volunteer for the Program. It did so under its prerogative so that it and the private parties it authorized to participate could enjoy the rich cash infusion the Program promised them. However, once the State agreed to participate in the Program willfully, it surrendered its right to fail to perform the duties of the Program and to administer the program outside the Equal Protection and Due Process Fourteenth Amendment requirements.

It is black letter law that the Program, 21 U.S.C. §360bbb-3, its FWA, and the EUA letters required the State to ensure its citizens were afforded the lawful authority to either accept or refuse the administration of the emergency use of the federal COVID-19 property. Moreover, the Supremacy Clause governing 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) and the express preemption language of the PREP Act restricted the State and any legal entity within the state from establishing conditions conflicting with the lawful rights of Plaintiffs to either accept or refuse the product. The U.S. Congress completely preempts the entire field of emergency use authorized products under 21 U.S.C. §360bbb-3 and "covered countermeasures" under the PREP Act.

Shriners Hospitals for Children, a private party authorized by Washington State to conduct the State's business of administering the federally funded COVID-

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19 drugs, was bound as an "Organization"[8] to inform any person under its authority of their lawful authority to accept or refuse the product. That legal right is held by Plaintiffs, not Defendants, and Defendants cannot cite a Constitution, statute, regulation, or proclamation, conferring upon them lawful authority to interfere in Plaintiffs' chosen option, either to accept or refuse. Moreover, because Defendants were acting under color of law on behalf of the State of Washington, they were bound to ensure Plaintiffs were treated equally before the law and in accordance with due process, which they did not when terminating Plaintiffs' employment when they chose the option to refuse. Therefore, Defendants were bound to statutory duties, contractual obligations, and Constitutional requirements when administering the Program on behalf of the State.

Shriners PolicyMakers were under the authority and directive of the State, which volunteered to administer the federal government's property. Moreover, the federal government warned Shriners PolicyMakers that "Other possession, distribution, delivery, administration, shipment receipt, or use of COVID-19 vaccine **outside the parameters of the Program** constitutes, at a minimum, theft under 18

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[8] Defendants are Shiners PolicyMakers (citation to defendants) and are lawfully bound to ensure the organization complies with its lawful obligations.

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U.S.C. § 641, and violation of other federal civil and criminal laws." (Emphasis added).

Finally, the Governor created State Policy under Proclamation 21-14.1 prohibiting "Any Health Care Provider from failing to be fully vaccinated against COVID-19 after October 18, 2021." On October 18, 2021, no drug licensed by the FDA, according to the product's labeling that was also introduced into commerce for general commercial marketing, existed within the State of Washington, nor did such a drug exist through the effective period of the Proclamation. Therefore, all actions taken under the policy by Defendants to fulfill the Proclamation were in violation of federal law, and Plaintiffs' statutory and constitutional rights and the contested actions can be "fairly attributed to the State" *Lugar* v. *Edmondson Oil Co.*, 457 U.S. 922, 935, 937 causing Defendants to become State Actors relating to the contested actions.

B. The Public Function Test

The public function test: "when private individuals or groups are endowed by the State with powers or functions governmental in nature, they become agencies or instrumentalities of the State and subject to its constitutional limitations." *Evans v. Newton,* 382 U.S. 296, 299, 86 S.Ct. 486, 15 L.Ed.2d 373 (1966) The State,

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traditionally and exclusively,[9] issues licenses to both medical facilities and healthcare workers. Moreover, the State determines the conditions and requirements under which that property will be issued and maintained. The State also performs the public function of managing emergency declarations, and such activities are exclusively under the State's prerogative. Lastly, only the State had the authority to determine how the public function of distributing the federal government's COVID-19 EUA drug property would be exercised, who could participate, and the conditions under which they could participate. Plaintiffs had a Fourteenth Amendment right to choose one of the two options under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) without consequence, and the State and Defendants were bound by the Fourteenth Amendment, 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III), the CDC COVID-19 Vaccination Program, their FWA agreements, and the EUA letters to inform Plaintiffs of their lawful rights to accept or refuse participation and to treat each option equally and in accordance with due process. Moreover, Defendants willfully participated in the State Action. However, Proclamation 24-14.1 deprived Plaintiffs of their legal interest under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) to refuse, treated them differently than those who chose to accept and stripped them of their substantive due process rights to seek judicial relief should they incur injury from

[9] *Marsh v. Alabama,* 326 U.S. 501, 505-06, 66 S.Ct. 276, 90 L.Ed. 265 (1946)

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the use of a listed countermeasure under the PREP Act such as Defendants drugs listed under a compulsory mandate.

The Public Function Test is met by (1) Shriners exercising powers exclusively held by the State,[10] (2) the power is part of the State's prerogative[11] (the State did not have to volunteer to participate, and Shriners PolicyMakers operate exclusively under the State's authority), and (3) the power is such that the State itself is obligated to perform due to Governor Inslee voluntarily agreeing to perform for the CDC.[12]

"Ownership does not always mean absolute dominion. . . . [Where] facilities are built and operated primarily to benefit the public . . . their operation is essentially a public function . . . subject to state regulation."[13] The use of Shriners Hospital for Childrens' organization serves a public function when they administer the federal COVID-19 property to the public and when they act upon emergency conditions issued by the State for the public's benefit. The State stated that its emergency

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[10] *Jackson v. Metropolitan Edison Co*., 419 U.S. at 352

[11] *Flagg Bros., Inc. v. Brooks*, 436 U.S. at 160. See also *Jackson v. Metropolitan Edison Co*., 419 U.S. at 353

[12] *Jackson v. Metropolitan Edison Co*., 419 U.S. at 353

[13] *Marsh v. Alabama* (1946)

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declaration was meant to serve the public (1) "WHEREAS, to protect some of our most vulnerable populations—persons in health care facilities, long-term care facilities (which includes nursing homes), and similar congregate care facilities—and to protect our health and congregate care systems themselves, I issued several proclamations imposing heightened protections on workers, residents, and visitors in those facilities," (2) "WHEREAS, it is the duty of every employer to protect the health and safety of employees by establishing and maintaining a healthy and safe work environment and by requiring all employees to comply with health and safety measures," (3) and as such, require "Any Health Care Provider from failing to be fully vaccinated against COVID-19 after October 18, 2021." Therefore, the function was the regulation of the emergency order under the Proclamation for the public's benefit.

The case "turns on what is quintessentially an exclusive and traditional public function" *Lee v. Katz*, 276 F.3d 550, 556-57 (9th Cir. 2002). Therefore, the State issuing emergency healthcare orders for the public's benefit is an exclusive and traditional public function of the State. Defendants regulated the employment of Plaintiffs on behalf of the State for purposes of serving a public function by requiring them to inject the federally owned COVID-19 property, which itself was a program under the State's prerogative serving a public function. "We have, of course, found state action present in the exercise by a private entity of powers traditionally

exclusively reserved to the State." *Jackson v. Metropolitan Edison Co.*, 419 U.S. 345 (1974)

C. The Symbiotic Relationship And Joint Activity Test

"A state becomes responsible for a private party's act if the private party acts (1) in an exclusively state capacity, (2) for the state's direct benefit, or (3) at the state's specific behest. It acts in an exclusively state capacity when it 'exercises powers traditionally exclusively reserved to the state[,]' 419 U.S. 352, 95 S.Ct. 454; for the state's direct benefit when it shares the rewards and responsibilities of a private venture with the state, see id., 357-58, 95 S.Ct. 456-57, *Burton v. Wilmington Parking Authority*, 365 U.S. 715, 723-24, 81 S.Ct. 856, 860-61, 6 L.Ed.2d 45 (1961); and at the state's specific behest when it does a particular act which the state has directed or encouraged." (*Modaber v. Culpeper Memorial Hospital, Inc.*, 674 F.2d 1023 4th Cir. 1982)

Therefore, these tests are met by Defendants (1) affirming they acted on behalf of Governor Inslee's Proclamation (State Policy) when discharging Plaintiffs refusing the administration of the federally funded property, (2) administered the COVID-19 Program on behalf of the State to share in the Program's cash flow benefiting the State and Defendants which required informing individuals of their rights as a condition of the Program's process, (3) agreed to obtain the informed consent of individuals on behalf of the state which means the State and Defendants

were required to create a legally approved environment ensuring Plaintiffs were under no outside pressure to participate in the Program's administration, (4) conducted joint research activities required of the federal government for each EUA drug, and (5) the State engaged in the joint action of administering the federal government's COVID-19 property and fulfilling Proclamation 24-01.

Shriners PolicyMakers (Defendants) are duly authorized to conduct the affairs of the "Organization" and issue Shriner's COVID-19 Vaccine policy, empower others to execute the policy, and enforce the policy. The Ninth Circuit Court of Appeals held that "A person 'subjects' another to the deprivation of a constitutional right, within the meaning of section 1983, if he does an affirmative act, participates in another's affirmative acts, or omits to perform an act which he is legally required to do that causes the deprivation of which complaint is made" *Johnson v. Duffy*, 588 F.2d 740, 743 (9th Cir. 1978). Defendants failed to perform the acts required of them under the CDC COVID-19 Vaccination Program as a State Actor and under the EUA letters subject to 21 U.S.C. §360bbb-3, and under the Fourteenth Amendment. Moreover, they specifically issued, enforced, and encouraged Shriners Hospitals for Children's Policy, which is the direct cause of injury to Plaintiffs, and thus, Defendants are subject to § 1983 enforcement.

The Ninth Circuit also applies a "Governmental compulsion" test, "Governmental compulsion or coercion may exist where the State 'has exercised

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coercive power or has provided such significant encouragement, either overt or covert, that the choice must in law be deemed to be that of the State.' *Blum v. Yaretsky* , 457 U.S. 991, 1004, 102 S.Ct. 2777, 73 L.Ed.2d 534 (1982)" *Rawson v. Recovery Innovations, Inc*., 975 F.3d 742, 748 (9th Cir. 2020). Defendants affirm they acted under government compulsion when executing Proclamation 24-14.1 and discharging Plaintiffs from employment.

The *Lugar* court, discussing similar facts, determined, "The statutory scheme obviously is the product of state action, and a private party's joint participation with state officials in the seizure of disputed property is sufficient to characterize that party as a 'state actor' for purposes of the Fourteenth Amendment. Respondents were, therefore, acting under color of state law in participating in the deprivation of petitioner's property." *Lugar v. Edmondson Oil Co., Inc*., 457 U.S. 922 (1982) The *Lugar* court concluded that the private party acting in concert with the State official under a state policy meant the private party was a state actor. The facts in this case are identical insofar as Defendants engaged in joint action with Governor Inslee under a State policy (Proclamation 24-14.1) in the deprivation of Plaintiffs' Equal Protection and Due Process rights when denying them their legal interest under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) and being treated unequally under the law. Moreover, the products were also listed as "covered countermeasures," requiring Plaintiffs to consent to the forfeiture of their due process rights to seek judicial relief

from resulting injuries. As such, the compulsory requirement of the State policy to participate in those products stripped Plaintiffs of their due process rights which act was a violation of their substantive due process rights. Therefore, Defendants' actions caused constitutional violations, and such actions were directly attributed to the State, causing Defendants to become state actors when engaging in the contested actions.

However, the Ninth Circuit makes it clear that "At bottom, the inquiry is always whether the defendant has 'exercised power' possessed by virtue of state law and made possible only because the wrongdoer is clothed with the authority of state law.'' *West v. Atkins*, 487 U.S. 42, 49, 108 S.Ct. 2250, 101 L.Ed.2d 40 (1988) (quoting *United States v. Classic* , 313 U.S. 299, 326, 61 S.Ct. 1031, 85 L.Ed. 1368 (1941)). *Rawson v. Recovery Innovations, Inc.*, 975 F.3d 742, 748 (9th Cir. 2020). Defendants were clothed with the power of the State to engage in the federally funded program under the State's direct authority and under the State's Proclamation. Defendants' claim that they are not State Actors fails across any legal lens upon which their actions are viewed.

However, because Defendants, like all other medical facilities within the State, had full legal authority to deny the application of the State Policy to the use of the federally owned COVID-19 drugs, Plaintiffs also assert that Defendants acted under State Custom. 42 U.S.C. § 1983 states, "Every person who, under color of any

statute, ordinance, regulation, **custom**, or usage…subjects, or causes to be subjected, any citizen of the United States or other person within the jurisdiction thereof to the deprivation of any rights, privileges, or immunities secured by the Constitution and laws, shall be liable to the party injured in an action at law…" (emphasis added) The Supreme Court noted in *Adickes v. S. H. Kress & Co.*, 398 U.S. 144 (1970) that the "Petitioner will have established a claim under §1983 for violation of her equal protection rights if she proves that she was refused service by respondent because of a state-enforced custom…" (emphasis added).

All without fear of consequence from the State, Shriners Hospitals for Children: (1) assured the federal government they would never place an individual under outside pressures to participate in a federally funded research activity via their FWA agreement nor under their contract with the Program (State Policy) but did so via their vaccine requirement; (2) signed the CDC COVID-19 Vaccination Program Provider Agreement promising to ensure Plaintiffs were given the option to accept or refuse, but removed that option via their Policy; (3) was required to obtain Plaintiffs' legally effective informed consent, but willfully ignored that duty; (4) was

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assigned an Institutional Review Board[14] requiring them never to place Plaintiffs under involuntary conditions when involved in INDs, but required compulsory participation under threat of penalty; (5) has no authority to amend the Scope of Authorization for each COVID-19 EUA drug, but amended the conditions by which the Scope of Authorization is lawful; (6) had no authority to establish conditions contrary to the CDC regarding its administration of the federally owned property, but ignored its duties under 12(a) of the Provider Agreement; (7) had no authority to establish conditions contrary to applicable requirements of any drug under an EUA, but ignored those applicable requirements; and (8) is restricted by the U.S. Congress from using any laws of the State or political subdivision of the State to interfere in the federal government's property by terminating Plaintiffs employment exercising a federally secured right to refuse participation in the product's administration, but used the State's at-will laws and the State custom to retaliate against Plaintiffs' choice of their federally secured option.

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[14] An IRB is required when Shriners Hospitals for Children (1) conducts a clinical trial under 21 CFR Parts 50&56, which is not the case at bar, (2) administers an investigational new drug to only one patient under any provision of 21 U.S.C. §360bbb et. seq., which is the case at bar.

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Therefore, Defendants deprived Plaintiffs of their legal rights by acting on a State-enforced custom having the force of law outside of any lawful authority. Governor Inslee, Attorney General Bob Ferguson, and Shriners PolicyMakers, as a State Actor, established and enforced a state custom whereby a person's EUA and PREP Act statutory rights to refuse the federal property could be ignored "as if" they did not exist and that they could be treated differently under the Fourteenth Amendment than persons choosing the option to accept the administration of the federal property.

Plaintiffs were refused access to living wages and demoted to second-class citizenry because a State custom had a force of law overriding the authority of the United States Constitution. Moreover, the Court held, "Based upon the language of the statute [sic] legislative history, and judicial decisions, the words 'under color of a . . . custom or usage, of [a] State,' in § 1983, mean that the 'custom or usage' must have the force of law by virtue of the persistent practices of state officials. (*Id*. Pp. 398 U.S. 162-169.") Although *Adickes* involved the state custom of racial discrimination, the precise custom is not the relevant point but rather the persistence of the custom or usage, no matter its name. The *Adickes* Court referenced one original Congressional supporter of § 1983, who stated, "[T]he chief complaint is not that the laws of the State are unequal, but that, even where the laws are just and equal on their face, yet, by a systematic maladministration of them, or a neglect or

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refusal to enforce their provisions, a portion of the people are denied equal protection under them." The Court commented, "This interpretation of custom recognizes that settled practices of state officials may, by imposing sanctions or withholding benefits, transform private predilections into compulsory rules of behavior no less than legislative pronouncements." Governor Inslee and Defendants, acting under a State-enforced custom, usurped the federal government's authority, imposed sanctions, and withheld benefits as if that purported authority had legislative pronouncement but in reality was only *ultra vires* authority.

Plaintiffs were discharged from employment for violating a state-enforced custom, not federal or state law, nor Shriner's vaccine policy, which might only be considered lawful when using FDA-approved drugs

At all times pertinent, Governor Inslee could have issued an executive order requiring agency enforcement of State obligations required of Shriners PolicyMakers and other authorized private parties conducting state business, but he did not. At all times pertinent, Attorney General Bob Ferguson could have utilized the enforcement powers of his office to correct Shriners PolicyMakers and private parties the state authorized to administer the federal property but did not. Shriners Policymakers acted on this State-encouraged and enforced custom when engaging in their unlawful conduct, which led to the deprivation of Plaintiffs' statutory and Constitutional rights. "Misuse of power, possessed by virtue of state law and made

possible only because the wrongdoer is clothed with the authority of state law, is action taken 'under color of' state law." *United States v. Classic*, 313 U.S. 299 (1941), *citing Ex Parte Virginia*, 100 U. S. 339, 100 U. S. 346; *Home Telephone & Telegraph Co. v. Los Angeles*, 227 U. S. 278, 227 U. S. 287, *et seq*.; *Hague v. CIO*, 307 U. S. 496, 307 U. S. 507, 307 U. S. 519; cf. 101 F.2d 774, 790.

As a matter of law, Shriners PolicyMakers could only establish a COVID-19 Vaccination Policy because the State agreed to administer the federal COVID-19 property in which it authorized Defendants to participate. Therefore, the State and Shriners PolicyMakers both conspired under a state custom to deprive Plaintiffs of their Constitutional and statutory rights and privileges because they were clothed with the authority of State Power.

Therefore, Plaintiffs dispute each claim described within Defendants motion to dismiss when they assert or imply they were not a state actor and or not engaged in state action when referring to the contested actions.

V. COMIRNATY v PFIZER-BIONTECH COVID-19 VACCINE

Defendants' defense is built around a logical fallacy that the legally distinct drug, Pfizer-BioNTech COVID-19 Vaccine, is the same as Comirnaty® for purposes of law because the two allegedly share the same formulation. As a matter of law, the drug's formulation has no relevance to the issues presented in Plaintiffs' Complaint because federal drug laws govern labels, not formulations. Once a label is applied to

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a drug vial, then that drug is eternally governed by that label, irrespective of a label attached to a different drug vial that might share the same formulation.

Comirnaty® is the FDA-approved[15] marketing name used on vials of Pfizer's FDA-approved COVID-19 drug called Comirnaty®, but not for the "Pfizer-BioNTech COVID-19 Vaccine." Only vials containing the formulation processed under the licensing requirements can bear the label "Comirnaty®." Comirnaty® is subject to strict standards, laws, and regulations governing how it can be manufactured, where, and what legal indications it can be prescribed and advertised for.[16] Unlike "Pfizer-BioNTech COVID-19 Vaccine," Comirnaty® had to meet the exacting standards under 42 U.S.C. § 262 and 21 U.S.C. § 355 before receiving marketing approval. This fact reveals why Comirnaty® had to receive an EUA for

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[15] FDA-approved means the label bears the name, indication, and usage approved under 21 USC § 355(a). Any drug vial not labeled as "Comirnaty®" is not approved for general commercial marketing and cannot operate under the laws governing an approved drug.

[16] Legal indication means the FDA-approved intended use of the drug. This includes factors such as age, gender, contraindications, warnings, etc. A drug cannot be marketed for an indication the FDA did not approve it for.

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its unapproved use (age indication) despite sharing formulation with its unapproved labeled EUA drug. Should Pfizer promote the use of Comirnaty® for an indication (age) that it is not approved for simply because it shares formulation with its EUA version, then such promotion is deemed misbranded, constituting a felony.[17]

Pfizer-BioNTech COVID-19 Vaccine is an investigational new drug under IND 19736 and was authorized only for emergency use pursuant to 21 U.S.C. §360bbb-3. Comirnaty® must be manufactured only at sites listed on its BLA, whereas the EUA drug must be manufactured at sites designated by the federal government, and the two manufacturing processes are not identical in nature. The EUA drug cannot bear the Comirnaty® label and vice versa. Should Defendants

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[17] In 2009, the FDA charged Pfizer with a felony for promoting four drugs outside of their legal indication and fined them over $1.19 billion. The Department of Justice noted, "Under the provisions of the Food, Drug and Cosmetic Act, a company must specify the intended uses of a product in its new drug application to FDA. Once approved, the drug may not be marketed or promoted for so-called "off-label" uses – i.e., any use not specified in an application and approved by FDA. - Justice Department Announces Largest Health Care Fraud Settlement in Its History. Justice.gov. Published September 2, 2009. Accessed June 6, 2023.

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place the Comirnaty® label on a vial containing the Pfizer-BioNTech COVID-19 Vaccine, they would be subject to civil and criminal prosecution. Should Pfizer treat its unapproved EUA drug as approved, then it too would be subject to civil and criminal prosecution. Comirnaty® cannot come under the EUA statute for its licensed use,[18] whereas the Pfizer-BioNTech COVID-19 Vaccine can never come under any licensed use. The medical profession is allowed to use Comirnaty® for "off-label" uses, whereas they can never use Pfizer-BioNTech COVID-19 Vaccine for uses not intended under the emergency declaration. Pfizer-BioNTech COVID-19 Vaccine is not identical to Comirnaty® because a drug is more than its formulation; it is manufacturing, regulations, treaties, import/export laws, federal laws, state laws, insurance regulations, and a plethora of other legal factors all attached to one indisputable fact, its labeling. Comirnaty® has a different label than the Pfizer-BioNTech COVID-19 Vaccine investigational new drug, and those labels are attached to entirely different sets of laws for very specific purposes as established by valid acts of Congress spanning 120 years of legislative activity. Moreover, once an emergency declaration has ended, the Pfizer-BioNTech COVID-19 vaccine

[18] 21 U.S.C. §360bbb-3(c)(3)

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cannot be utilized in the marketplace as an approved drug, nor can the Comirnaty® label be attached to the product.[19]

Nearly all US States have misbranding laws, such as Washington State under RCW 69.04.016. Misbranding laws are applicable not only to the labeling on the vial but also to the boxes in which the vials are shipped and any documentation, promotion, and advertising associated with the product.[20] Defendants were careful not to state that the EUA drugs were approved by the FDA, only that Pfizer's two drugs shared formulation. However, the wording was designed to mislead, not to inform the end user, that the legal distinctions between the two drugs have legal consequences for the end user, as previously discussed. (Doc 1, ¶ 163).

Therefore, as a matter of law, all vials of the Pfizer-BioNTech COVID-19 Vaccine were manufactured, distributed, administered, and regulated under laws governing the use of unapproved drugs. On August 23, 2021, when FDA approved the marketing application for Comirnaty®, the laws applicable to Pfizer-BioNTech COVID-19 Vaccine remained the same. Nothing changed for the EUA version since it was not manufactured, distributed, or administered under the BLA requirements

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[19] 21 U.S.C. §360bbb-3(b)(2)(B) disposition defined under 21 U.S.C. § 360eee(4)

[20] 21 U.S.C. § 321

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of Comirnaty®. The FDA, CDC, HHS, and all other persons continued under the laws governing the unapproved use of the Pfizer-BioNTech COVID-19 Vaccine despite another legally distinct drug sharing the same formulation being approved on paper. The FDA did not treat the Pfizer-BioNTech COVID-19 Vaccine drug differently after it approved COMIRNATY for general commercial marketing. Only Defendants treated the drug differently and in violation of previously discussed federal law.

Should this court rule that the two drug labels are not legally distinct, even though such a ruling would conflict with FDA's own documentation and Congressional requirement, it would have to enjoin the FDA, CDC, HHS, and the entire medical community from treating Pfizer-BioNTech COVID-19 Vaccine as an investigational new drug, which means it would also have to enjoin the HHS from issuing additional EUAs for the COVID-19 pandemic and to terminate all current EUAs. Moreover, it would require writing legislation and voiding volumes of U.S. Code, drug regulations, and international laws governing their import and export. Though such a statement appears nonsensical at first blush, this is precisely what Defendants are requesting of this court in their Motion to Dismiss. The entire $600 billion pharmaceutical industry operates under federal law requiring their products to be regulated according to their labeling. Essentially, Defendants ask this court to rule that drug labeling laws no longer have effect when stating that "The Pfizer-

BioNTech vaccine authorized for EUA use and Comirnaty® were identical vaccines, and accordingly a fully FDA-authorized vaccine was publicly available to individuals 16 or older as of August 23, 2021"(Doc 32, p. 19, lines 4-6).

For the reasons set forth in Footnote 2 of Shriners' Motion to Dismiss (Doc 32, p.4), Plaintiffs move this court to take judicial notice of the declaration provided by Peter Marks, Director of the Center for Biologics Evaluation and Research ("CBER"), United States Food and Drug Administration ("FDA"), in *Coker v Austin*, stating that "Vaccines are approved for marketing through applications known as Biologics License Applications ("BLA"); a vaccine that is the subject of an approved BLA need not also obtain approval of a new drug application ("NDA") under 21 U.S.C. § 355. 42 U.S.C. § 262(a), (j)."[21] "Additionally, FDA determined that '[t]he licensed vaccine has the same formulation as the EUA-authorized vaccine and the products can be used interchangeably to provide the vaccination series without presenting any safety or effectiveness concerns.'" "FDA provided this information in the authorization letter to make clear that pharmacies and other healthcare practitioners could provide the vaccination series to recipients using Pfizer- BioNTech, Comirnaty®, or both (e.g., first dose of Pfizer-BioNTech followed by second dose of Comirnaty®, or vice versa), since the formulation was

[21] See Exhibit B, Declaration of Peter Marks

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the same and both products are made by the same manufacturer under current good manufacturing practice requirements. FDA included the interchangeability information in the authorization letter to avoid the unnecessary operational complications that may have resulted if pharmacies or other healthcare practitioners had believed that individuals who had received Pfizer-BioNTech for the first dose were not authorized to receive Comirnaty® for the second dose, or vice versa. While FDA determined Comirnaty® and the Pfizer-BioNTech Covid- 19 vaccine **are medically interchangeable**, **there are legal distinctions between BLA-approved and EUA-authorized products.** For example, products approved under BLAs are required to have the labeling that was approved as part of the BLA, whereas products authorized under the EUA would have the EUA labeling, and there may also be differences in manufacturing sites for the BLA and EUA vaccine." (Emphasis added). This fact is why the FDA informed the public in its August 23, 2021 EUA letter (See Exhibit A, EUA Letter from FDA to Pfizer, August 23, 2021) that Pfizer's two products "are legally distinct."

Therefore, it is a logical fallacy for Defendants to claim that because Comirnaty® shares the same formulation with Pfizer-BioNTech COVID-19 Vaccine, they also share the same labeling operating under the same laws. Defendants want the court to rule in this manner because it is the hinge upon which they defend their unlawful activity. If the Pfizer-BioNTech COVID-19 Vaccine

investigational new drug was lawfully considered an approved drug, then Defendants are implying that the HHS Secretary violated the duties of his office by continuing issuing EUAs which Congress restricts if a FDA-approved product meeting the intended emergency use exists within the marketplace. Moreover, should a person seek legal action against Pfizer for an adverse event, the court would not, and could not, allow a Plaintiff to use laws associated with Comirnaty® if the Plaintiff was administered Pfizer-BioNTech COVID-19 Vaccine simply because it shared formulation with Comirnaty® because the two drugs do not share the same laws. Such facts reveal the nonsensical nature of Defendants' claim. However, Defendants are legally sophisticated in drug labeling laws since they dispense them hourly across the nation. Moreover, they have been involved in millions of dollars of medical research for decades using the same laws governing the Pfizer-BioNTech COVID-19 Vaccine drug. Plaintiffs cannot cite to a federal statute affording them authority to dispense an investigational drug outside of the strict federal guidelines, none of which allows them to treat the investigational new drug classification as an approved drug according to the product's labeling. However, Defendants do present their claim as being "plainly incompetent," which nullifies their qualified immunity claim (*Malley v. Briggs*, 475 U.S. 335, 341 (1986)). Therefore, Plaintiffs dispute each allegation by Defendants when they claim or imply that they could mandate compulsory use of the FDA-classified investigational new drug, Pfizer-BioNTech

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COVID-19 Vaccine, under their vaccine requirement because it shared formulation with Comirnaty®.

VI. 21 U.S.C. §360bbb-3 UNDER § 1983

Defendants assert throughout their motion that "courts have consistently held that there is no private right of action to assert Section 1983 claims under the EUA Act (21 U.S.C. § 360bbb-3)" (Doc 32, p. 12). First, "private right of action" is not synonymous with "§1983 cause of action." Second, Plaintiffs are not aware of any court holding that 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) is not enforceable under § 1983, and Defendants cited to no cases to support their claim. Defendants cite cases holding that there is no private right of action under the statute to seek judicial relief, which is not the same thing as saying that there is no action under 42 U.S.C. § 1983 for violations of the rights created for Plaintiffs under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) by a valid act of Congress. It is well-established U.S. Supreme Court precedent that a federal statute conferring a right onto an individual in unambiguous language is enforceable under 42 U.S.C. § 1983 (*Maine v. Thiboutot*, 448 U.S. 1 (1980)). 21 U.S.C. §360bbb-3 creates a right for individuals considering participation to either accept or refuse the products under autonomous conditions free from outside pressures. The statute does not create a right for Defendants to interfere in the freely chosen option of Plaintiffs. Rather, it restricts that interference by restricting the Secretary from having the authority to require any person to

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participate in any activity the Secretary authorizes. Defendants claim that 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) is not enforceable under a § 1983 cause of action fails. Therefore, Plaintiffs dispute each allegation by Defendants when they assert that 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) is not enforceable under § 1983.

VII. QUALIFIED IMMUNITY

Defendants assert a claim that they are entitled to qualified immunity, which is incorrect. Private persons sued under 42 U.S.C. § 1983 are not entitled to qualified immunity as government employees. The Supreme Court supports this claim, and the Ninth Circuit does not generally grant qualified immunity to private parties.

The Ninth Circuit discussed in detail in *Jensen v. Lane County*, 222 F.3d 570 (9th Cir. 2000) how and why the Supreme Court holds that private parties are not entitled to qualified immunity. In the *Jensen* case, the Ninth Circuit held that a private physician contracting with a county hospital was a state actor but was not entitled to qualified immunity.

Shriners cannot assert a good faith qualified immunity argument because they operate under the investigational new drug classification laws when used under 21 U.S.C. §360bbb *et seq*., 21 CFR Parts 50, 56, 45 CFR Part 46, and the Belmont Report and have been doing so for decades and know, or should have known, that the investigational new drug classification can never come under compulsory use. Shriners also cannot be granted qualified immunity because their duties and

responsibilities under the applicable laws were "clearly established" because Shriners knew, as affirmed in their Motion to Dismiss, the contractual requirements of the CDC COVID-19 Provider Agreement and do not dispute that they have a FWA and IRB governing the drug's classification.

Citing *Saucier v. Katz*, 533 U.S. 194, 201 (2001), provides no help to Shriners because Plaintiffs have asserted plausible constitutional violations which a reasonable person in Defendants' position would know to be a violation at the time the violation occurred. This fact is why Defendants assert the logical fallacy that simply because Comirnaty® was approved, they could then treat the "Pfizer-BioNTech COVID-19 Vaccine" as an FDA-licensed drug despite providing no federal statutes or case law supporting that position. Instead, Shriners hangs their qualified immunity defense on the fact that they issued a "vaccine mandate" while not speaking to the gravamen of the case, which is how they required the use of EUA/PREP Act drugs for compliance with their mandate. Moreover, the drugs were fully funded by the federal government, and thus the State specifically required Shriners to abide by all applicable laws regarding the drug's labeling which prohibited compulsory use.

VIII. RESPONSE TO DEFENDANTS' OTHER ARGUMENTS

Defendants' assertion that "Plaintiffs are 12 former Shriners Children's employees discharged after they chose not to receive a COVID vaccine" is incorrect.

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Plaintiffs were discharged for no other reason than exercising their lawful authority under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) to refuse injection of federally funded investigational drugs, not a licensed vaccine. (Doc 32, p. 11)

Defendants' assertion that "a fully authorized vaccine was available at the time of Shriners Children's requirement" (Doc 32, p. 12) is misleading because it fails to distinguish between "authorization" and "approved." Pfizer-BioNTech COVID-19 Vaccine was authorized for emergency use purposes, and Comirnaty® was approved for general commercial marketing purposes (i.e., FDA-licensed). But only one was actually "available." Absolutely zero vials of the FDA-licensed Comirnaty® were available to Plaintiffs at all times pertinent. This is a factual allegation that must be accepted as true. It would be error for the court to take judicial notice that Comirnaty® was approved and then assume it was also "available" to Plaintiffs because (a) Plaintiffs dispute Defendants' assertion that Comirnaty® was introduced into commerce for general commercial marketing and will prove otherwise on Motion for Summary Judgment or trial, and (b) there is no government document in existence establishing that Comirnaty® was "available" at any time during 2021 or 2022, as is demonstrated by Defendants providing no evidence to support its allegation that Comirnaty® was available to Plaintiffs. Moreover, Plaintiffs can, and will, produce irrefutable evidence from Pfizer, Inc., FDA, CDC, and declaration statements issued in federal courts by the Department of Justice

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proving that Comirnaty®'s original formulation, was never manufactured by Pfizer, Inc. and its second licensed formulation, Tri-Sucrose, was also not manufactured and distributed into the marketplace for general commercial marketing in the entire nation. Defendants provide nothing to show when, where, and how Comirnaty® was available at any time before the Governor's October 18, 2021 deadline. Defendants only requested the court to take judicial notice that the FDA approved Comirnaty®, not that the drug actually shipped.

Defendants' assertion that "Vaccination requirements are lawful, do not implicate fundamental rights, and need only satisfy the deferential rational basis review standard" (Doc 32, p. 13) is not applicable to this action. This case does not involve a vaccination requirement. It involves an EUA/PREP Act drug mandate. Thus, while requirements to take an FDA-licensed vaccine may require the rational basis standard, requirements to relinquish one's fundamental right of bodily autonomy and integrity upon an order to be injected with an unlicensed EUA/PREP Act drug would certainly require the strict scrutiny standard.

Defendant's assertion that "Plaintiffs' claims that vaccine requirements violate constitutional rights" (Doc 32, p. 13) is patently false, and no such claim exists nor is implied within their Complaint. It is only when Defendants assert that they have the right to mandate the use of an EUA/PREP Act drug that Plaintiffs claim such a requirement would violate Constitutional rights. To be clear, Plaintiffs

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only asserted that the loss of their legal interest under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) and the PREP Act to refuse the product is a violation of their fundamental constitutional and federal statutory rights. Defendants cannot cite to Plaintiffs claiming otherwise.

Defendants' citation to *Biden v. Missouri*, 595 U.S. 87, 93 (2022) (Doc 32, p. 14) is misplaced. In that case, the Supreme Court only answered the question of whether the President had the authority to issue a vaccine mandate, not whether he had the authority to mandate the use of an EUA/PREP Act drug. Instead of citing *Biden v. Missouri*, it is more appropriate to cite to another admittedly uncontrolling case but one that is closest to being on point with the case at bar. *Doe v. Rumsfeld*, 297 F. Supp 2d 119 (DC 2003) and 341 F. Supp 2d 1 (DC 2004), is a DC Circuit case addressing a mandate that DoD personnel (service members and civilians) be vaccinated with the anthrax vaccine, which was an EUA investigational drug at the time. The court held that persons under DoD authority cannot be forced to take an investigational drug as a condition of their employment. That court held: The Court is persuaded that the right to bodily integrity and the importance of complying with legal requirements that may potentially be inconvenient or burdensome, are among the highest public policy concerns one could articulate.

Defendants' assertion that, "[o]n December 11, 2020, the U.S. Food and Drug Administration (FDA) issued the first emergency use authorization (EUA) for a

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COVID vaccine" (Doc 32, p. 16) is misleading. The FDA issued an EUA to Pfizer for a product named "Pfizer-BioNTech COVID-19 Vaccine" but clarified that the drug was "an investigational vaccine not licensed for any indication." (Doc 1, p. 16)

Defendants' assertion that "Shriners Hospitals for Children does not have the ability to adapt or vary from the statements within the [Governors vaccine] proclamation" (Doc 32, p. 17) is misleading. Shriners Hospitals for Children could not lawfully vary from a Governor's proclamation relating to licensed drugs, but it could, and must, vary from a proclamation that Shriners require persons, under threat of penalty, to have federally owned investigational products injected into their bodies, especially when the Governor and Shriners previously assured the federal government they would not require any person to inject such drugs under threat of penalty. Moreover, Governor Inslee and Defendants were free to comply with their ministerial duties when the Proclamation stated, "the specific prohibitions in this Proclamation are severable and do not apply to the extent that compliance with a prohibition would violate (1) any U.S. or Washington constitutional provision; (2) federal statutes or regulations; (3) any conditions that apply to the state's receipt of federal funding; (4) state statutes; or (5) applicable orders from any court of competent jurisdiction." The unlawful actions taken by Defendants demonstrate that they were only acting under a state-enforced custom. Additionally, Defendants' use of the CDC COVID-19 Vaccination Program and associated drugs were under

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federal law, superseding Governor Inslee's lawful authority to interfere in the federal statutes in the manner in which he interfered.

Defendants mischaracterize a District of New Mexico's ruling in *Legaretta v. Macias*, 603 F. Supp. 3d 1050, 1061 (D.N.M. 2022)(Doc 32, p. 19), which in turn mischaracterizes *Doe v. Rumsfeld*. (Doc 32, p. 19) The *Legaretta* court did what Shriners Defendants are attempting to do in the case at bar – confuse the two products as being one and the same. The only "Pfizer COVID-19 vaccine" that is FDA-licensed is Comirnaty® but it was never available to Plaintiffs before the October 18, 2021 deadline. The "Pfizer-BioNTech COVID-19 Vaccine" was never FDA-licensed (only EUA/PREP Act approved) but was always available. These are irrefutable facts that are proven by the products' labeling, and thus, it would be erroneous for this court to hold that the two products are one and the same and are subject to the same laws.

Throughout the motion, Defendants attempt to use the fact that an emergency existed to justify their actions. (Doc 32, pp. 19-21) However, Congress enacted the EUA and PREP Act statutes under the knowledge that they would only be used in times of an emergency and still restricted, in unambiguous language, the compulsory use of products authorized under those statutes.

Defendants assert that the Plaintiffs were not subjected to "investigational drug use" because they never received a COVID vaccine fails because the option to

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accept or refuse, by definition, must be given to a person before being injected with the drug. (Doc 32, p. 31). Defendants would have this court believe that one must be injected with an EUA drug in order to be able to exercise one's right to refuse the injection. This is the type of absurd result courts have in mind when discussing the inapplicability of absurd arguments.

Moreover, Defendants seem to be confused about the legal meaning of "subjected" (Doc 32, p. 31). Black's Law Dictionary (2nd Ed) defines "subject to" as "conditional or dependent on something" or "being under domination as of an authority or government." Therefore, Defendants' claim that Plaintiffs "were never 'subjected' to anything" is patently false. As soon as Plaintiffs were told that they were being offered an EUA/PREP Act drug, they were "subjected to" an EUA/PREP Act product and their right to refuse such drugs immediately vested. Instead, though, Shriners Defendants placed a condition upon Plaintiffs to inject unlicensed investigational EUA/PREP Act drugs into their bodies to maintain employment, be treated equally, and enjoy the option to refuse without fear as other employees choosing the option to accept. Thus, Plaintiffs were subjected to investigational drug use by Defendants.

Defendants seem to be confused as to what constitutes medical experimentation for purposes of Article VII of the ICCPR Treaty. (Doc 32, p. 31) Plaintiffs defined the statutory meaning of medical experimentation and the history

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behind that meaning (Doc 32, FN 61, FN 62, see Section V). None of the cases cited by Defendants knew of the broad meaning of medical experimentation or research (Doc 1, ¶ 37). The definition is very broad for a specific purpose, and that purpose is to protect any person involved in a federally funded investigational new drug. Unlike Plaintiffs' Complaint, Defendants do not define "medical experimentation" or "research," so there is no basis for understanding why they disagree with the sources of authority Plaintiffs cite defining the terms of those words and how those terms apply to Plaintiffs' claims.

Defendants' quotation (Doc 32, p. 32) of the *Bridges* case is irrelevant here because the Plaintiffs did not assert that Shriners engaged in a human trial nor attempted to put them under a clinical trial. A clinical trial requiring the use of an IRB is governed under 21 CFR 312.50-312.70.

Defendants' statement that the "Plaintiffs' claim also fails given that the COVID vaccine at issue was not…'investigational' as they allege" (Doc 32, p. 32) is incredulous since they are disputing the FDA, not an opinion offered by Plaintiffs. (See Doc 1, Footnotes 21 and 22)

Defendants' assertion that "10 U.S.C. section 980, the other federal statute that Plaintiffs cite, does not provide a basis for a claim against Defendants" is incorrect. (Doc 32, p.5) While that statute may appear to apply only to the military, a close reading reveals that the statute requires informed consent from **any person**

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involved in research activities under DoD funding. The DoD procured 100m doses of Pfizer-BioNTech COVID-19 Vaccine using its authorized funding and distributed those doses, among others, throughout the states under the CDC research conditions discussed in Plaintiffs' complaint. Therefore, the DoD funding provides Plaintiffs with a § 1983 cause of action because Defendants did not allow Plaintiffs to exercise their lawful right to give informed consent as defined under 45 CFR § 46.116.[22] The Belmont Report and 45 CFR § 46.116 are the only two authoritative legal sources defining the meaning of informed consent involving investigational new drugs.

Defendants' assertion that 45 CFR Part 46 is not enforceable under a § 1983 claim is incorrect. (Doc 32, p. 26) As Plaintiffs alleged (Doc 1, Section V), 45 CFR Part 46 was developed at the specific request of the U.S. Congress[23] to protect humans involved in biomedical research activities after much national attention was given to the abuses by the medical industry when involving humans in investigational drugs such as the case at bar. It contains specific duties of any person involving others in federally funded investigational new drugs and provides specific rights to those considering such participation. Through the use of the FWA, all US States and Territories, Counties, major hospitals, universities, and all federal

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[22] *Health and Hospital Corp of Marion Cty. V. Talevski, 599 U.S. ___ (2023).*

[23] National Research Act, 1974.

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agencies and departments are bound the regulations and the Belmont Report. (Doc 1, ¶ 56) Moreover, Defendants in this case have an FWA agreement as a condition to receive federal funding or participate in federally authorized research activities such as the CDC COVID-19 Vaccination Program. However, through Shriner's FWA and its voluntary agreement in the Program, a § 1983 cause of action exists because Defendants agreed to comply with the regulation and the Belmont Report when participating in the fully funded CDC COVID-19 Vaccination Program involving investigational COVID-19 EUA drugs fully funded by the federal government and only distributed through its authority.[24]

Defendants' claim that Shriners did not violate Plaintiffs substantive and procedural due process rights fails for reasons described herein and in the Complaint. (Doc 32, p. 27) Moreover, Defendants' attempt to attach the Plaintiffs' claim to a vaccine mandate relying on licensed drugs fails because the drugs to which Plaintiffs were subjected were not licensed drugs, and Defendants have brought forth no evidence that FDA-licensed COVID-19 drugs were available before the October 18, 2021 deadline imposed on Plaintiffs.

Defendants' claim that Plaintiffs' substantive due process violation claim does not implicate fundamental rights is incorrect because they rely on the logical fallacy

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[24] *Health and Hospital Corp of Marion Cty. V. Talevski, 599 U.S. ___ (2023).*

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that Pfizer's two drugs are legally the same. (Doc 32, p. 29) As discussed herein, that position is incorrect and unsupported by any evidence. Plaintiffs' allegations that the drugs are legally distinct must be accepted as true on a Motion to Dismiss.

Defendants' citation to the *Jacobson* case is not applicable for the reason that there are specific laws governing the drugs in question, which was not the case in *Jacobson*. (Doc 32, p. 30) Bodily autonomy and bodily integrity are fundamental rights. Requiring Plaintiffs to have their skin pierced with a needle containing an EUA/PREP Act drug violates those fundamental rights. Thus, there is a fundamental right to refuse the administration of any drug labeled by the FDA as investigational, and Congress has ensured that fundamental right is secured by expressly preempting Defendants from engaging in the unlawful actions described in the complaint. (Doc 32, p. 41) Shriners can point to no compelling state interest to warrant such a violation of fundamental rights, especially when COVID-19 has a 99.95% survival rate, the COVID-19 injections do not prevent infection or transmission, but they do result in serious adverse events such as myocarditis, blood-clotting, strokes, heart attacks, and sudden death.

Plaintiffs dispute Defendants' claim that their vaccination requirement should be "evaluated under the same rational basis" as a lawful vaccine mandate. (Doc 32, p. 41) Defendants issued a vaccine mandate but unlawfully required the use of drugs that Congress specifically restricts from coming under compulsory requirements; a

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fact Defendants knew or should have known intimately. Plaintiffs assert that their actions should be evaluated under the strict scrutiny basis.

As Defendants' cited on page 34 of Doc 32, "To prevail on an equal protection claim, a plaintiff must 'show that a class that is similarly situated has been treated disparately.'" *Williams*, 567 F. Supp. 3d at 1227 (quoting *Boardman v. Inslee*, 978 F.3d 1092, 1117 (9th Cir. 2020)). (Doc 1, p. 44) Plaintiffs were healthcare professionals who were treated differently based on their 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) option, depriving them of their fundamental Equal Protection rights under the Fourteenth Amendment.

Defendants do not speak to the claims asserted by Plaintiffs regarding the PREP Act, except so simply say on page 40 of Doc 32 that it is an immunity statute that does not apply because Plaintiffs have alleged no physical injury, even though Plaintiffs did allege "emotional distress; mental, psychological and physical harm; and loss of enjoyment of life" in addition to various economic injuries. (Doc 1, ¶ 411) Instead, they attempt to build an argument distracting from the real argument. First, only the PREP Act can establish what is a covered countermeasure. Second, the PREP Act expressly preempted the State and Defendants from establishing conditions interfering with Plaintiffs 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) option as discussed. Third, Plaintiffs allege that the compulsory requirement under Defendants' mandate of covered countermeasures prospectively strips Plaintiffs of

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their legal interests to pursue judicial relief should they sustain injury from the countermeasure or any activity related to that countermeasure which is a deprivation of Plaintiffs' substantive due process rights. This fact is why the PREP Act requires only voluntary participation, which Defendants violated in their vaccination requirement. The basis for a § 1983 cause of action under the PREP Act is contained within the express preemption language and the voluntary requirements of the statute. The PREP Act preempted the State and private parties it authorized to administer the federally owned EUA drugs from interfering in the Plaintiffs' free choice under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III). Those rights are specifically created by the PREP Act in favor of the Plaintiffs and anyone else subjected to the use of a PREP Act product.

Defendants fail in their claim that the CDC COVID-19 Vaccination Program Provider Agreement does not provide third-party beneficiary rights. (Doc 32, p. 52) The entire Provider Agreement was designed to protect the potential recipient of the drugs, and the applicable laws provide third-party rights, specifically the right to accept or refuse without penalty or financial requirement.

Defendants' statement that "Regardless of the entity that signed on Shriners Children's behalf, the Provider Agreement existed between the organization and the CDC—not any of the individually named Shriners Children's Defendants. Because none of the individually named Shriners Children's Defendants were parties to the

Provider Agreement, Plaintiffs' breach of contract claim against them must be dismissed" ignores the following actual language from the Provider Agreement:

> For the purposes of this agreement, in addition to Organization, Responsible Officers named below will also be accountable for compliance with the conditions specified in this agreement. The individuals listed below must provide their signature after reviewing the agreement requirements.

The individuals listed below are the Chief Medical Officer and the Chief Executive Officer Therefore, Defendants' argument fails.

Defendants' claim that Plaintiffs did not state any viable claim alleging a Washington State common law employment tort is patently false. (Doc 32, p. 55-57) Plaintiffs stated the claim and provided case law to support the claim. (Doc 1, ¶ 400). Moreover, the term "Shriners PolicyMakers" was referenced 75 times in the Complaint, describing their actions and how those actions violated Plaintiffs' statutory and constitutional rights. As described herein and in the Complaint, Shriners PolicyMakers enacted the vaccination policy, authorized persons to enforce the policy, and the policy was the basis for Plaintiffs being deprived of their rights, leading to financial, emotional, and other damages.

Defendants' claim that Plaintiffs' Outrage claim should be dismissed for reasons they instituted a vaccine requirement fails for reasons previously stated. (Doc 32, p. 58)

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IX. ORAL ARGUMENT REQUESTED

Plaintiffs believe oral argument would be helpful and hereby request same.

X. CONCLUSION

For the reasons set forth above, Plaintiffs urge the court to deny PeaceHealth's Motion to Dismiss, in its entirety.

RESPECTFULLY SUBMITTED,

Date: 1/5/24

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